

D344

The Assessment and Diagnostic Process of Psychiatric Nurse Practitioner Practice

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

At a first visit, a 40-year-old graphic designer says, "I don't even know why I'm here; my wife made me come." The office has new furniture and soft lighting. Which response by the PMHNP is most therapeutic?

- A. "Don't worry, everyone feels that way at first."
- B. "Let's make you comfortable first; do you like the new chairs?"
- C. "Why don't you want to be here?"
- D. "Tell me more about what led your wife to suggest coming in."

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A patient in a red sweater says, "I feel fine, really good today," while speaking in a flat, monotone voice with little facial expression and tearful eyes. She arrived ten minutes early. How should the PMHNP record the MSE?

- A. Mood: "fine, really good"; affect: constricted and incongruent with stated mood.
- B. Affect: bright, based on her red sweater and punctuality.
- C. Mood: depressed; affect: "fine."
- D. Mood and affect both euthymic, since she says she feels good.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

Which statement in an MSE note describes affect rather than mood?

- A. Patient states, "I'm so anxious I can't think."
- B. Thought process linear and goal-directed.
- C. Labile, shifting quickly from laughing to crying during the interview.
- D. Oriented to person, place, and time.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 58-year-old recently divorced man with chronic back pain says he feels like a burden to his family. He owns a hardware store and has two adult children. What should the PMHNP do next?

- A. Ask directly whether he has thoughts of suicide, a plan, intent, and access to means.
- B. Reassure him that his children obviously love him.
- C. Refer him to pain management only.
- D. Avoid the word suicide so the idea is not suggested.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A patient who drove 40 minutes to the clinic completes the PHQ-9. Items 1-8 are scored 2, 2, 1, 3, 1, 2, 1, 2, and item 9 is 0. She says she has had a hard year. What are the total score and severity?

- A. Severe, because she had a hard year
- B. 1.6, minimal
- C. 14, moderately severe
- D. 14, moderate

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A patient's PHQ-9 falls from 18 at baseline to 8 after 12 weeks of treatment. How should this change be classified?

- A. Remission.
- B. A 10% improvement.
- C. No response, because symptoms remain.
- D. Treatment response, but not remission.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

On an inpatient unit painted in calming blue, patients attend a daily community meeting, share responsibility for unit tasks, and follow a posted schedule of groups. The unit has 18 beds. What best describes this approach?

- A. Individual psychotherapy.
- B. Milieu therapy, using the structured environment and community as part of treatment.
- C. Environmental design; the blue paint is the treatment.
- D. Seclusion-based behavior management.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A 26-year-old woman who works at a coffee shop has frequent intense mood swings, chronic self-harm when upset, and unstable relationships. She has tried two SSRIs. Which psychotherapy is best matched to her main problem?

- A. Psychoeducation alone
- B. A third SSRI trial instead of therapy
- C. Dialectical behavior therapy
- D. Motivational interviewing

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A patient says, “Part of me wants to cut back on drinking, but it’s how I relax.” Which approach fits this statement best?

- A. Motivational interviewing
- B. Exposure therapy
- C. Dialectical behavior therapy
- D. Confronting the patient about denial

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A community mental health clinic that opened a second site last year tracks the percent of new patients screened with the PHQ-9 at intake. The clinic's director also serves on the county board. In the Donabedian model, what type of measure is this?

- A. Structure
- B. Process
- C. Governance
- D. Outcome

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

Which quality measure is an outcome measure in the Donabedian model?

- A. Any number reported to the board.
- B. The number of licensed PMHNPs on staff.
- C. The percent of patients with a 50% or greater drop in PHQ-9 score at 12 weeks.
- D. The percent of patients screened with the PHQ-9 at intake.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A PMHNP licensed only in Ohio starts a scheduled video visit with an established patient, who mentions she is calling from a hotel in Florida while on a work trip. She has used the clinic's portal for two years. What should the PMHNP do?

- A. Proceed, since the patient has used the portal for years.
- B. Confirm location and follow licensure rules; reschedule or arrange care unless Florida permits it, while handling any emergency.
- C. Proceed, because the PMHNP's Ohio license covers established patients anywhere.
- D. Never see patients by video if they could travel.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

At a first visit, ~~a 40-year-old graphic designer~~ says, "I don't even know why I'm here; my wife made me come." ~~The office has new furniture and soft lighting.~~ Which response by the PMHNP is most therapeutic?

Target word: best interview response **Decides:** Open-ended invitation that respects his view.

A	Adjacent category	False reassurance closes exploration.
B	Camouflage	Focuses on the setting, not the patient.
C	Confusion set	Why questions sound judgmental and put patients on the defensive.
D	Correct	Open-ended and invites his perspective.

2. Answer A Concept 2

A patient ~~in a red sweater~~ says, "I feel fine, really good today," while speaking in a flat, monotone voice with little facial expression and tearful eyes. ~~She arrived ten minutes early.~~ How should the PMHNP record the MSE?

Target word: mood vs. affect **Decides:** Mood is her report; affect is observed.

A	Correct	Records her words as mood and the observation as affect.
B	Camouflage	Clothing and arrival time do not define affect.
C	Inverted operation	Swaps the two elements.
D	Dropped element	Ignores the observed affect.

3. Answer C Concept 2 · transfer

Which statement in an MSE note describes affect rather than mood?

Target word: reverse: identify affect **Decides:** Affect is an observation.

A	Inverted operation	Patient's report is mood.
B	Adjacent category	That is thought process.
C	Correct	An observed pattern of emotional expression.
D	Confusion set	That is cognition/sensorium.

4. Answer A Concept 3

A 58-year-old recently divorced man with chronic back pain says he feels like a burden to his family. ~~He owns a hardware store and has two adult children.~~ What should the PMHNP do next?

Target word: next step in risk assessment **Decides:** Burden statements are a warning sign requiring direct inquiry.

A	Correct	Direct questioning is standard and does not increase risk.
B	Camouflage	Uses the family detail for reassurance rather than assessment.
C	Dropped element	Leaves out suicide risk assessment.
D	Inverted operation	Asking directly does not plant the idea.

5. Answer D Concept 4

A patient ~~who drove 40 minutes to the clinic~~ completes the PHQ-9. Items 1-8 are scored 2, 2, 1, 3, 1, 2, 1, 2, and item 9 is 0. ~~She says she has had a hard year.~~ What are the total score and severity?

Target word: total and band **Decides:** Sum = 14.

A	Camouflage	Severity comes from the score, not the comment.
B	Inverted operation	Averages the items instead of summing them.
C	Adjacent category	Moderately severe starts at 15.
D	Correct	2+2+1+3+1+2+1+2+0 = 14; 10-14 is moderate.

6. Answer D Concept 4 · transfer

A patient's PHQ-9 falls from 18 at baseline to 8 after 12 weeks of treatment. How should this change be classified?

Target word: response vs. remission **Decides:** 18 to 8 is a 56% drop; 8 is not below 5.

A	Adjacent category	Remission requires a score under 5.
B	Wrong stem number	Misreads the 10-point drop as a percentage.
C	Absolute language	Response does not require zero symptoms.
D	Correct	A drop of 10 points (≥50%) with a score still above 4.

7. Answer B Concept 5

On an inpatient unit ~~painted in calming blue~~, patients attend a daily community meeting, share responsibility for unit tasks, and follow a posted schedule of groups. ~~The unit has 18 beds.~~ What best describes this approach?

Target word: milieu therapy **Decides:** Structured environment and community as treatment.

A	Adjacent category	These are group and environmental interventions.
B	Correct	Structure, shared roles, and meetings define the milieu.
C	Camouflage	Decor alone is not the milieu.
D	Inverted operation	Milieu promotes engagement, not isolation.

8. Answer C Concept 6

A 26-year-old woman ~~who works at a coffee shop~~ has frequent intense mood swings, chronic self-harm when upset, and unstable relationships. ~~She has tried two SSRIs.~~ Which psychotherapy is best matched to her main problem?

Target word: modality match **Decides:** Emotion dysregulation with self-harm.

A	Dropped element	Lacks skills training for dysregulation.
B	Camouflage	Prior medication trials do not change which therapy fits.
C	Correct	DBT targets emotion regulation, distress tolerance, and self-harm.
D	Confusion set	MI addresses ambivalence about change.

9. Answer A Concept 6 · transfer

A patient says, “Part of me wants to cut back on drinking, but it’s how I relax.” Which approach fits this statement best?

Target word: reverse: from problem to modality **Decides:** Ambivalence about change.

A	Correct	MI explores and resolves ambivalence.
B	Adjacent category	Exposure treats anxiety and trauma.
C	Confusion set	DBT targets emotion dysregulation.
D	Inverted operation	Confrontation increases resistance.

10. Answer B Concept 7

A community mental health clinic ~~that opened a second site last year~~ tracks the percent of new patients screened with the PHQ-9 at intake. ~~The clinic’s director also serves on the county board.~~ In the Donabedian model, what type of measure is this?

Target word: classify the measure **Decides:** Screening is something clinicians do.

A	Adjacent category	Structure covers resources like staffing or sites.
B	Correct	It measures a care activity.
C	Camouflage	Not a Donabedian category; draws on the board detail.
D	Inverted operation	Outcomes are patient results, such as symptom improvement.

11. Answer C Concept 7 · transfer

Which quality measure is an outcome measure in the Donabedian model?

Target word: reverse: find the outcome **Decides:** A result for patients.

A	Absolute language	Reporting does not make a measure an outcome.
B	Adjacent category	A resource: structure.
C	Correct	Symptom change is a patient result.
D	Inverted operation	A care activity: process.

12. Answer B Concept 8

A PMHNP licensed only in Ohio starts a scheduled video visit with an established patient, who mentions she is calling from a hotel in Florida while on a work trip. ~~She has used the clinic’s portal for two years.~~ What should the PMHNP do?

Target word: licensure and location **Decides:** Licensure follows the patient’s location.

A	Camouflage	Portal history does not affect licensure.
B	Correct	The clinician must be authorized where the patient is located.
C	Inverted operation	Licensure is tied to the patient’s location.
D	Absolute language	Telehealth is appropriate when location is verified and licensure fits.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	8	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	9	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	4	
Camouflage	Answers a stem fact that was never the question.	8	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Therapeutic interviewing	Open-ended questions, reflection, and empathic listening build rapport; why questions, false reassurance, and advice-giving block it.	Choosing reassurance or a why question because it sounds supportive.	Choosing the best interviewer response.
2	Mental status exam: mood vs. affect	Mood is the patient's own report of emotional state (often quoted); affect is the clinician's observation of emotional expression (range, congruence).	Recording the clinician's observation as mood.	Patient's words vs. what the clinician sees.
3	Suicide risk assessment	Ask directly about suicidal thoughts, plan, intent, means, and history; asking does not plant the idea; respond with a risk formulation and safety plan.	Avoiding direct questions for fear of suggesting suicide.	Hopelessness, a positive screen, or a recent loss.
4	PHQ-9 scoring and response	Sum nine items scored 0–3 (0–27): 5 mild, 10 moderate, 15 moderately severe, 20 severe; response = at least 50% drop; remission = score below 5; any positive item 9 needs direct suicide assessment.	Averaging items or confusing response with remission.	Item scores or a baseline and follow-up total.
5	Milieu therapy	The therapeutic milieu uses the structured unit environment, routines, and community interactions as part of treatment.	Equating milieu with room decor or free, unstructured time.	Unit rules, community meetings, or structured daily schedules.
6	Matching therapy modality to need	CBT targets distorted thoughts and behaviors; DBT builds emotion regulation and distress tolerance (e.g., chronic self-harm, borderline personality disorder); motivational interviewing resolves ambivalence about change.	Choosing a familiar therapy rather than the one matched to the problem.	The core problem described: thoughts, emotion dysregulation, or ambivalence.
7	Donabedian quality framework	Structure = resources and setting; process = what clinicians do; outcome = the result for patients.	Calling a process measure (e.g., screening rate) an outcome.	A quality measure to classify.
8	Telehealth safety and licensure	At the start of each telehealth visit confirm the patient's current physical location and an emergency contact; the clinician must be licensed where the patient is located.	Assuming the clinician's own state license covers any patient location.	A video visit with a patient who may be traveling.

Course scope (public description): The Assessment and Diagnostic Processes for Advanced Psychiatric Mental Health Practice guides students when examining determinants to the role of the psychiatric mental health nurse practitioner. This course guides students in building a therapeutic relationship with patients through interviewing skills, conducting a structured assessment, milieu, types of therapy, and various care strategies, including technology usage. This course guides students through exploring their leadership role in collaborating with the interprofessional community as a mental health nurse practitioner. Pathways of quality improvement, practice evaluation, and healthcare reform are also considered. The following courses are prerequisites: All MSN Core courses, Advanced Pathophysiology for the Advanced Practice Nurse, Advanced Pharmacology for the Advanced Practice Nurse, and Advanced Health Assessment for the Advanced Practice Nurse.